● ARCHIVE EDITION 03

VOICE PACK ARCHIVE

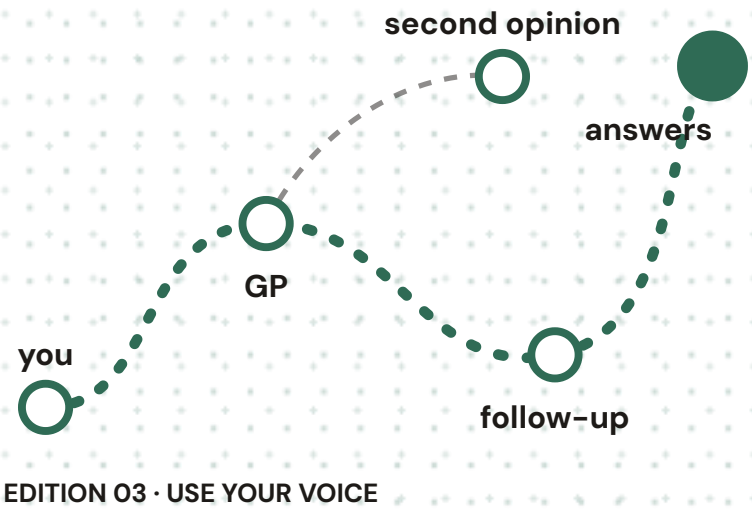
Use Your Voice

“What do I do when I’m not being heard?”

You can understand your body and describe it clearly, and still walk out of an appointment feeling brushed off. This edition is about what happens next: how to hold your ground, find your way through the system, and know your rights.

01 UNDERSTAND | 02 PREPARE | 03 COMMUNICATE | 04 NAVIGATE | 05 ADVOCATE

About 35 minutes to read For ages 14+



FROM THE ARCHIVE

By this edition, the stories young people were sharing with us had changed. Many had the knowledge and the words, and were still being told it was “just stress”, “normal for your age”, or nothing at all. We stopped treating dismissal as a gap in the young person’s skills and started naming it for what it is. This is where Aletheia’s self-advocacy and healthcare-navigation work took shape.

UNDERSTAND

01 When you’re not heard

Medical dismissal is when your concerns aren’t properly listened to, investigated or explained. It isn’t always rude or obvious. It can sound kind. It can come from someone who means well. The result is the same: you leave without answers, and start to doubt yourself.

It is not a test you failed. Knowing what it looks like makes it easier to respond in the moment, instead of replaying it on the bus home.

What it can look like

- Symptoms put down to stress, anxiety or weight before anything else is checked
- “That’s normal” without asking how it affects you
- Being told you’re too young for a condition
- Being interrupted, or the appointment wrapping up before you’ve finished
- Questions directed to your parent instead of you
- Leaving without a plan or any idea what happens next

Why it happens

- Short appointments and busy clinics
- Pain in girls and women being normalised for generations
- Assumptions about age, gender, culture, weight, disability or sexuality
- Language barriers, or no interpreter
- Symptoms that are hard to see or test for
- Young people not being expected to speak for themselves

> **Being dismissed says something about the conversation. It says nothing about whether your symptoms are real.**

Implicit bias

WHAT IT IS

Assumptions people hold without realising, which can shape whose pain is believed and what gets investigated. Research and public inquiries in Australia have found women's pain is more likely to be underestimated.

GOOD TO KNOW

Naming bias isn't about blaming one doctor. It helps you see why pushing for proper investigation is reasonable.

Diagnostic delay

WHAT IT IS

The time between first having symptoms and getting a diagnosis. For endometriosis in Australia it averages 6 to 8 years, partly because symptoms get normalised.

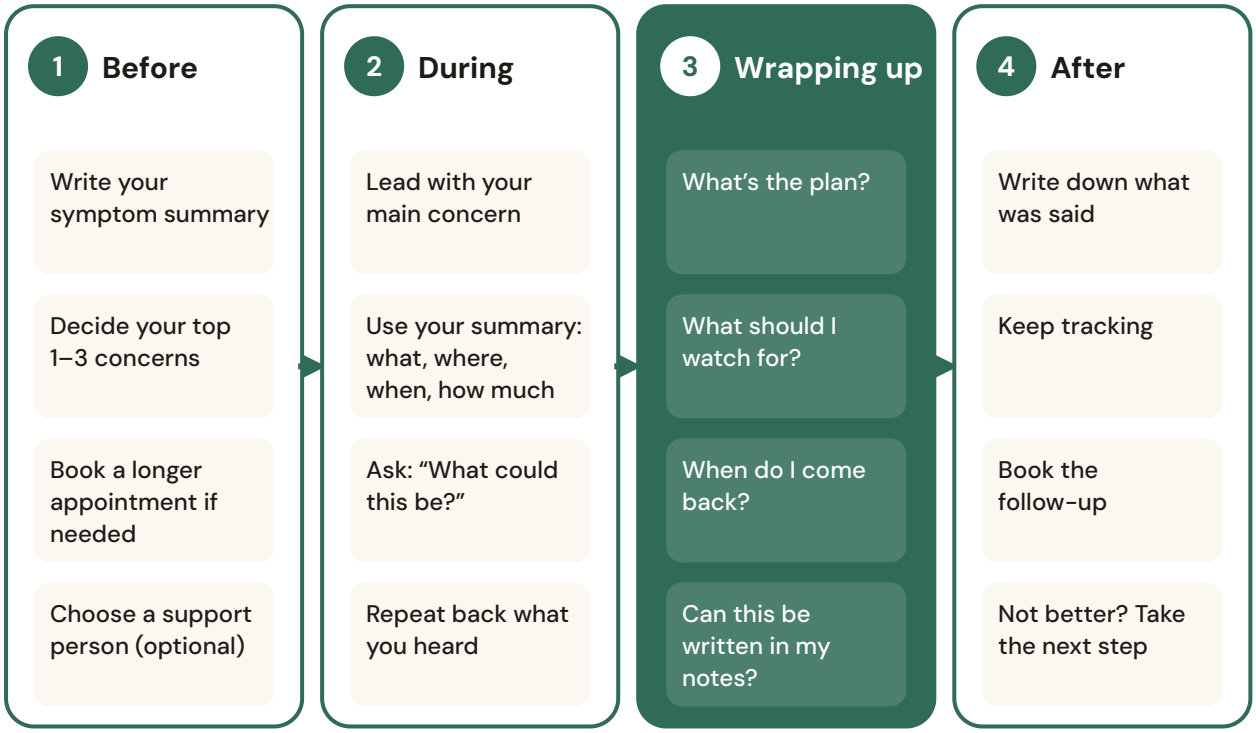
GOOD TO KNOW

Delay is a pattern, not a personal failure. Edition 04 looks at what can change it.

PREPARE

02 Walk in prepared

Preparation is the quiet superpower of self-advocacy. It means the important things get said even if you feel nervous or rushed.



Stage 3, wrapping up, is where the plan gets made. Don't skip it.

Your one-page summary

Bring a short page (or a note on your phone) with:

- Your main concern in one sentence
- Your answers to what, where, when, how much and what changes it
- How long it's been going on, and whether it's getting worse
- What you've already tried
- Past appointments about it, and what was said
- What you want from today

You can bring someone

A parent, carer, older sibling, friend or youth worker can come with you to take notes or back you up. You decide whether they stay for the whole appointment. You can also ask to speak to the doctor alone for part of it.

> "Can my friend stay while we talk about this?"

> "Could I have a few minutes on my own with the doctor?"

Choose the right kind of appointment

Book a long appointment for anything complex. Ask for a doctor you prefer (for example, a female GP) if that helps you speak openly. Telehealth works for follow-ups and talking through results. If you'd like an interpreter, ask when you book: it's free for GP appointments covered by Medicare.

COMMUNICATE

03 What to say in the room

You don’t need to be confrontational. The most effective responses are calm, specific, and bring the focus back to what the symptoms are doing to your life.

When it’s put down to stress

CLINICIAN *It’s probably just stress. Try to get more sleep.*

YOU Stress might be part of it. I’d still like to rule out a physical cause, because this pain has stopped me going to school four times this term.

YOU What would you want to check before we put it down to stress?

You’re not rejecting the idea. You’re asking for reasoning and a proper look.

When you’re told it’s normal

CLINICIAN *A lot of girls your age have painful periods. It’s normal.*

YOU I know period pain is common. But it isn’t manageable for me: it’s an 8 out of 10 and I have to lie down.

YOU At what point would you investigate it further?

When you’re interrupted

CLINICIAN *Right, so we’ll try some—*

YOU Sorry, there’s one more thing I need to mention before we decide.

NOTE Look at your notes. Finish the point. You’re not being rude by completing your sentence.

When you don’t understand

CLINICIAN

It could be secondary dysmenorrhoea. We’ll see how you go on the pill.

YOU

Can you explain that in simpler words? What would the pill be treating?

YOU

So, just to check: you think the pain might be caused by something else, and this is to see if it helps first. Is that right?

Repeating back what you heard (called ‘teach-back’) catches misunderstandings before you leave.

LINES THAT CHANGE THE CONVERSATION

> “ This is affecting my daily life, and I’d like to look into it properly.

> “ What else could this be?

> “ What’s the plan if this doesn’t work?

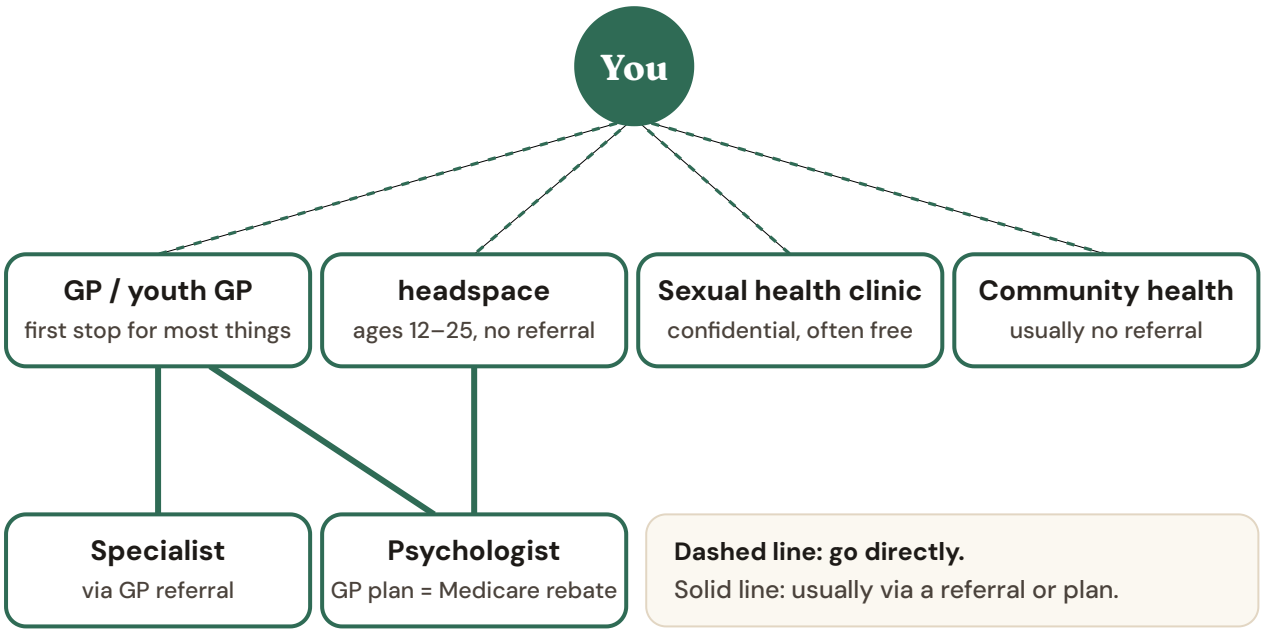
> “ Could you please note in my file that I raised this today?

> “ I’d like a referral, or a second opinion.

NAVIGATE

04 Finding your way through the system

Australia's health system has many doors. Knowing which one to use, and that there's always another, makes it much harder to get stuck.



QUICK OR URGENT HELP, STRAIGHT FROM YOU

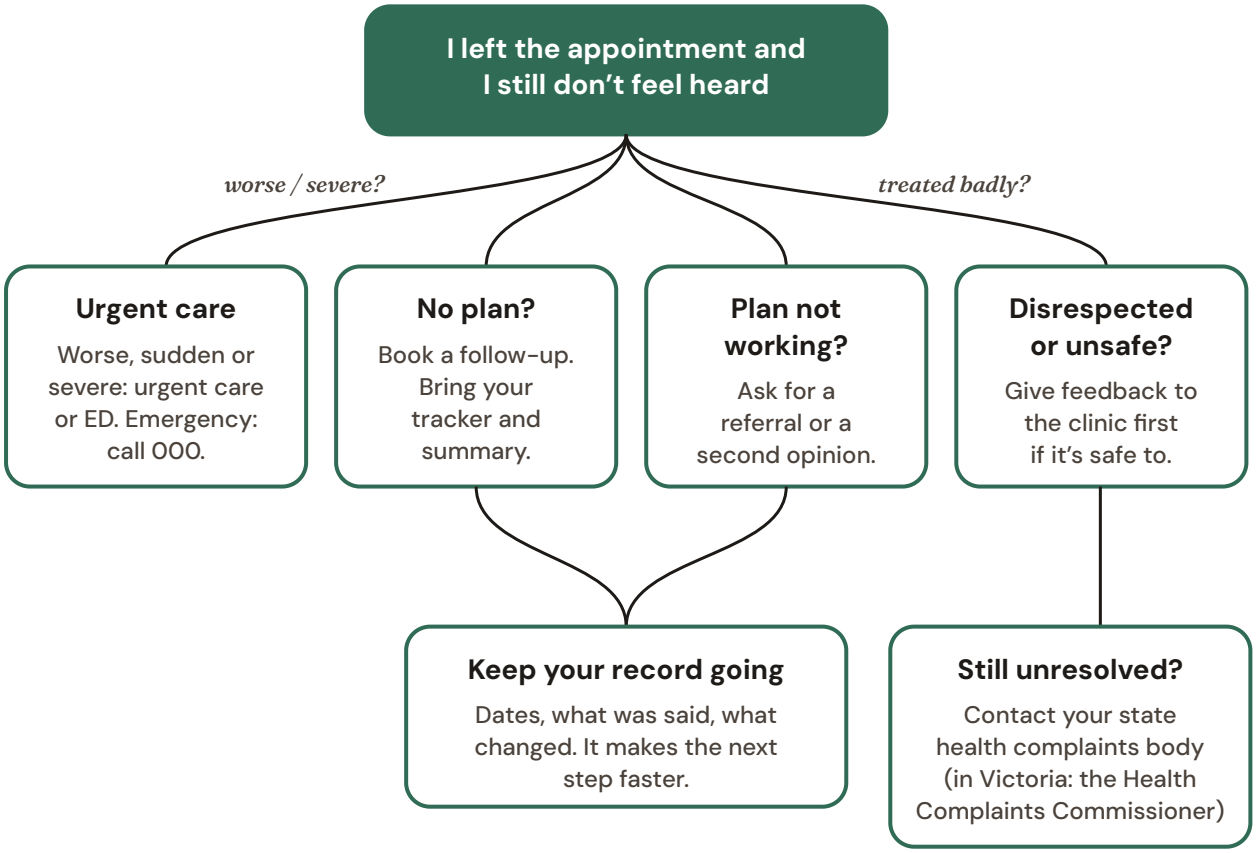
- **Pharmacist** — advice, no appointment
- **Urgent Care Clinic** — walk in, not life-threatening
- **Hospital ED / 000** — serious or emergency

You don't need a referral for a GP, headspace, a sexual health clinic, community health, a pharmacist or an Urgent Care Clinic.

IF YOU'RE DEALING WITH…	A GOOD FIRST STOP	GOOD TO KNOW
A physical symptom that's ongoing or worrying	GP	Ask if they bulk bill. A GP can refer you to a specialist.
Low mood, anxiety or "not feeling like myself"	GP, headspace or school wellbeing	A GP can make a Mental Health Treatment Plan for rebated psychology sessions.

IF YOU'RE DEALING WITH…	A GOOD FIRST STOP	GOOD TO KNOW
Contraception, STI testing, pregnancy questions	Sexual health clinic or GP	Often free and confidential. Some don't need a Medicare card.
Something urgent but not life-threatening	Urgent Care Clinic	Free walk-in care for things like sprains, cuts or infections.
Not sure how urgent it is	healthdirect, 1800 022 222	A registered nurse, 24/7, who can tell you where to go.
An emergency	Call 000 or go to ED	Severe pain, heavy bleeding with dizziness, trouble breathing, or thoughts of harming yourself.

When you leave still not feeling heard



Referrals

A referral is a letter from your GP to a specialist. It's needed to get a Medicare rebate for most specialists. Ask why they're referring you, whether there's someone closer or cheaper, and how long the referral lasts.

> “ Would it help to see a specialist about this?

> “ Is there a public clinic option with a shorter wait?

Second opinions

Seeing another doctor is a normal part of healthcare. You don't need permission, and you don't have to tell your first doctor. You can ask for a copy of your results to take with you.

> “ I'd like a second opinion.

> “ Could I have a copy of my test results?

ADVOCATE

05 Your rights, in plain words

The Australian Charter of Healthcare Rights applies to everyone getting healthcare in Australia, including young people. It has seven parts.

ACCESS

Healthcare that meets your needs.

SAFETY

Safe, high-quality care in a safe environment.

RESPECT

Being treated with dignity, and your culture, identity and beliefs respected.

PARTNERSHIP

Asking questions and being involved in decisions about your care.

INFORMATION

Clear information about your condition, options, costs and risks.

PRIVACY

Your personal and health information kept private and confidential.

GIVE FEEDBACK

Commenting on or complaining about your care, and having it dealt with.

Seven rights

They're yours at 14, 16 and 24 alike. Many clinics display the Charter in the waiting room.

Consent

Before any test or treatment, you should be told what it is, why it's recommended, the risks and the alternatives. You can ask questions, take time, say no, or change your mind.

Under 18? In most of Australia there's no single age. If a doctor thinks you understand the decision well enough (often called being a 'mature minor'), you can consent yourself.

Confidentiality

Doctors must keep what you tell them private, with some exceptions, such as a serious risk of harm to you or someone else. If you're worried, ask at the start.

> "Will this stay between us?"

> "Is there anything you'd have to share?"

From 14, Medicare generally needs your consent before your claims information is released to a parent, and at 15 you can get your own Medicare card.

If something went wrong: giving feedback

1. **Speak to the clinician or the practice**
   If it feels safe, raise it directly, or ask for the practice manager. Many issues are fixed here.
2. **Put it in writing**
   A short email or feedback form: what happened, when, and what you'd like done. Keep a copy.
3. **Contact your state health complaints body**
   If it's still not resolved. In Victoria this is the Health Complaints Commissioner, which is free. Every state and territory has one.

ADVOCATE

06 When culture, gender or identity are in the room

Who you are shapes how you're heard. Good healthcare adapts to you, not the other way round.

If English isn't your family's first language

You have the right to understand your care. GPs can book a professional interpreter for free through TIS National. A professional interpreter is better than a family member, especially for private topics.

> "I'd like an interpreter, please."

> "Can we speak without my family translating?"

If health isn't talked about at home

In many families, periods, sexual health and mental health are private or taboo. You can still get care. Confidential services exist for exactly this, and a school wellbeing worker can help you find one.

If you're LGBTQIA+

You have the right to respectful care. You choose what to share about your identity. It's okay to correct your name or pronouns, and to ask whether a service is inclusive. QLife (1800 184 527) offers peer support.

> "My name is ___ and I use ___ pronouns."

> "Is this service LGBTQIA+ inclusive?"

If you're Aboriginal or Torres Strait Islander

Aboriginal Community Controlled Health Organisations offer culturally safe care, run by community. 13YARN (13 92 76) is 24/7 crisis support by Aboriginal and Torres Strait Islander people.

If you're neurodivergent or have a disability

You can ask for adjustments: more time, a quieter room, written information, step-by-step explanations, or a support person.

> "I process information better in writing. Could you write that down?

If you're told you're too young

Some conditions, like endometriosis, can start in the teenage years. Age is a reason to be thorough, not a reason to stop looking.

> "I understand I'm young, but this is affecting me now. Can we look into it properly?

MAYA, TWO YEARS ON

Holding her ground

Maya's plan helped a little, then stopped helping. The pain has spread to between her periods. At a new appointment she's told it's "probably normal for her age".

This time she has her one-page summary and two years of tracking. She says calmly that it isn't manageable, and asks what else it could be. When she senses the appointment wrapping up, she asks what the plan is if nothing changes, and asks for her concerns to be noted in her file.

She leaves with a referral to a gynaecologist. On the way out, she books a follow-up anyway, just in case.

Maya is fictional. Her story continues in Edition 04.

REFLECT

Before your next appointment

1. What's the one thing I most need them to understand?
2. What will I say if I'm told it's normal?
3. Who could I bring, or tell about the appointment?
4. What would a good outcome look like for me?

WHERE THIS IDEA WENT NEXT

From one voice to the tools young people use now

Self-advocacy became one of Aletheia's foundations. It gave us the Medical Dismissal pillar, the Health Navigator, and the workshop young people ask for most.

Health Navigator & Self-Advocacy Journal
The full guide to getting care

Five Pillars
Understanding Medical Dismissal

Youth Workshops
"Heard in the Room": practising the scripts

Next: Archive Edition 04
When it isn't just you

Health education, not medical advice. This pack explains general health information so you can understand your body and talk about it. It cannot diagnose you. If you're worried about your health, speak to a doctor, nurse or other health professional. **In an emergency, call 000.**

Emergency · 000
If you or someone else is in danger or needs urgent medical help.

Kids Helpline · 1800 55 1800
Free, 24/7 phone and webchat, ages 5–25.

Lifeline · 13 11 14
24/7 crisis support. Text 0477 13 11 14.

headspace · headspace.org.au
Free or low-cost support, ages 12–25.

13YARN · 13 92 76
24/7 crisis support by Aboriginal and Torres Strait Islander people.

QLife · 1800 184 527
LGBTIQ+ peer support, 3pm–midnight.

healthdirect · 1800 022 222
24/7 nurse advice: is this urgent, and where should I go?

TIS National · 131 450
Free interpreting, when you need to talk to a service in your language.

▶ **Sources and fact-checking (10)**

Written and designed by The Aletheia Project resource development team. Scenarios are fictional. We acknowledge the Wurundjeri Woi-wurrung people of the Kulin Nation, where Aletheia was founded. Sovereignty was never ceded.